

Anxiety and Panic Attack - Male (Adult)

Algorithm 1061 | Adult (18 years and older) | Gender at birth: Male

Definition

- Panic attack/anxiety assessment decomposed from NHS.UK's published panic disorder guidance.
- Adult patient.
- Male.

Clinical background

- Panic attacks can cause sudden intense fear with physical symptoms, but emergency cardiac or breathing presentations must be excluded first.
- Repeated unexpected panic attacks followed by persistent worry may indicate panic disorder.
- A single settling episode without emergency features still requires safety-net advice.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant/recently postpartum, when did this episode start, and is it first, different, or more severe than prior clinician-diagnosed panic episodes?
2. [YES_NO] Has this happened before?
3. [OPEN_TEXT] Describe what the caller is feeling right now.

Triage and redirect questions

1. Redirect → Depression

Is persistent low mood or loss of interest the main concern, without a current panic episode?

Persistent depressive symptoms belong in the depression pathway; suicide risk must still be screened immediately.

1. Disposition level 100

Is there crushing or pressure-like chest pain, pain spreading to the arm/jaw, fainting, or severe difficulty breathing that does not fit a typical panic attack pattern?

Universal emergency rule-out added ahead of the panic disorder guidance itself (NHS.UK's page notes it does not give 999 criteria) - panic attack symptoms can closely mimic cardiac or respiratory emergencies, which must be ruled out first, not assumed to be anxiety.

1. Disposition level 50

Has the caller had regular, unexpected panic attacks followed by at least a month of ongoing worry about having more?

NHS.UK guidance: this pattern may indicate panic disorder and warrants a primary-care review for assessment.

1. Disposition level 15

Is this a single, isolated panic episode now settling, with none of the features above?

NHS.UK guidance provides in-the-moment coping techniques for an isolated panic episode.

Advice

Qatar emergency response for symptoms not safely attributable to panic

- Call Qatar emergency services on 999 and do not drive for first/severe chest pain, abnormal breathing, fainting, stroke signs, seizure, anaphylaxis, pregnancy/postpartum danger signs, poisoning, or immediate risk of harm. Keep the person with you, allow the position that makes breathing easiest, and do not assume a prior panic diagnosis explains new or different symptoms. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening chest pain or breathing; fainting, seizure, or neurological change; risk of self-harm or harm to others.

Routine panic disorder follow-up

- Book a routine primary-care review to discuss recurring panic attacks and treatment options.
- Call Back If: attacks become more frequent or severe.

Coping with a panic attack

- Stay where you are if possible, breathe slowly and deeply, remind yourself the attack will pass and is not life-threatening, and focus on calm, positive images.
- Call Back If: symptoms do not settle; chest pain or breathing difficulty develops.

First aid

- Call Qatar emergency services on 999 and do not drive for first/severe chest pain, abnormal breathing, fainting, stroke signs, seizure, anaphylaxis, pregnancy/postpartum danger signs, poisoning, or immediate risk of harm. Keep the person with you, allow the position that makes breathing easiest, and do not assume a prior panic diagnosis explains new or different symptoms. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Panic disorder", <https://www.nhs.uk/mental-health/conditions/panic-disorder/> (page last reviewed 22 August 2023)

Provenance

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